Appendix 1.

Plan to render an article.

Useful word combinations for rendering and discussion

1. The headline of the article. The author. The publication data. – Название статьи. Автор. Источник публикации.

The article I'm going to give a review of is taken from… (the website/ scientific journal) — Статья, которую я сейчас хочу проанализировать из…

The headline of the article is — Заголовок статьи…

The author of the article is… — Автор статьи…

It is written by — Она написана …(автором)

Unfortunately, the author of the article is not mentioned. – Автор статьи не упомянут.

The article is written on the 16th of November – Статья написана … (дата)

The article under discussion is … — Статья, которую мне сейчас хочется обсудить…

2. The topic of the article and logical parts.

The topic of the article is… — Тема статьи

The key issue of the article is… — Ключевым вопросом в статье является

The article under discussion is devoted to the problem… - Статья, которую мы обсуждаем, посвящена проблеме…

The article is about… - Статья о …

The article deals (is concerned) with… - В статье рассматривается

The article touches upon the issue of… - В статье затрагивается вопрос о

The purpose of the article is to give the reader some information on… - Цель статьи – дать читателю некоторую информацию о

The aim of the article is to provide the reader with some material on…- Цель статьи – предоставить читателю некоторый материал по

The article under discussion may be divided into several logically connected parts which are…the introduction, the main part and the conclusion —

Статья может быть разделена на несколько логически взаимосвязанных частей, таких как…(вступление, основная часть, заключение).

3. The content of the article. Summary. – Краткое содержание.

The author starts by telling the reader that — Автор начинает, рассказывая читателю, что

The author (of the article) writes (reports, states, stresses, thinks, notes, considers, believes, analyses, points out, says, describes, asserts, mentions, explains, outlines, generalizes, reveals, gives a summary, dwells on, exposes)

that… - Автор (статьи) пишет (сообщает, констатирует, подчеркивает, думает, отмечает, рассматривает, полагает, анализирует, указывает, говорит, описывает, утверждает, упоминает, поясняет, обрисовывает, обобщает, раскрывает, резюмирует, останавливается на , раскрывает), что…

The author draws the reader's attention to… - Автор обращает внимание читателя на

Much attention is given to… - Большое внимание уделяется…

According to … the scientists/ doctor Smith/ the rule/ the results … - Согласно ученым/ доктору Смиту/ правилу/ результатам (когда ссылаемся на кого-то, чьи-то слова или мнения)

The article goes on to say that… - Далее в статье говорится, что

It is reported (shown, stressed) that … - Сообщается (показано, подчеркнуто), что

It is spoken in detail about… - Подробно говорится о…

From what the author says it becomes clear that… - Из того, что говорит автор, становится ясно, что

The fact that … is stressed. - Тот факт, что... подчеркивается, акцентируется.

The article gives a detailed analysis of… - В статье дается подробный анализ

The author gives full coverage to… — Автор полностью охватывает…

The article contains the following facts…./ describes in details… — Статья содержит следующие факты …. / подробно описывает

Further the author reports (reports, states, stresses, thinks, notes, considers, believes, analyses, points out, says, describes, asserts, mentions, explains, outlines, generalizes, reveals, gives a summary, dwells on, exposes) that… / draws reader's attention to...

In conclusion the author writes (reports, states, stresses, thinks, notes, considers, believes, analyses, points out, says, describes, asserts, mentions, explains, outlines, generalizes, reveals, gives a summary, dwells on, exposes) that… / draws reader's attention to...

In conclusion the author says / makes it clear that…/ gives a warning that… — В заключение автор говорит / проясняет, что … / дает предупреждение, что …

The author comes to the conclusion that…- Автор приходит к выводу, что

4. Your opinion (conclusion) – Ваш вывод, мнение, отношение к вопросам, поднимаемой в статье.

Taking into consideration the fact that — Принимая во внимание тот факт, что

The main idea of the article is — Основная идея статьи (послание автора)

In addition… / Furthermore… — Кроме того

On the one hand…, but on the other hand… — С одной стороны …, но с другой стороны …

Back to our main topic… - Вернемся к нашей основной теме

To come back to what I was saying… - Чтобы вернуться к тому, что я говорил

In conclusion I'd like to… — В заключение я хотел бы …

From my point of view…/ In my opinion/ To my mind… — С моей точки зрения …

As far as I can judge/ see/ know/ understand… — Насколько я могу судить/

I fully agree with / I don't agree with -Я полностью согласен с/ - Я не согласен

It is hard to predict the future, but- Трудно предсказать будущее, но...

I have found the article informative/ entertaining/ dull / important / interesting /of great value because … - Я нахожу статью информативной/ развлекательной/ скучной / важной/ интересной/ имеющую большое значение (ценность), потому что…

Appendix 2.

Rendering an article

Symptoms and Diagnosis of PTSD

By Matthew Tull, PhD Updated on April 20, 2021

Post-traumatic stress disorder (PTSD) occurs after a traumatic event and interferes with a person's ability to function. You may wonder if you or someone you care about has PTSD, and whether you need to get professional help. If you are experiencing symptoms of PTSD, it is important to see a doctor so that you can get the right diagnosis and treatment.

The difference between PTSD and stress

Not everyone who has experienced a traumatic event will develop PTSD. After a traumatic event, it is normal to have strong feelings of anxiety, sadness, or stress. Some people may even experience nightmares, memories about the event, or problems sleeping at night, which are common characteristics of PTSD.

However, these symptoms do not necessarily mean that you have PTSD. Think of it this way: Headaches can be a symptom of a bigger problem, such as meningitis.

However, having a headache does not necessarily mean that you have meningitis. The same is true for PTSD. Many of the symptoms are part of the body's normal response to stress, but having them does not mean that you have PTSD.

DSM-5 Criteria for PTSD

Criterion A: Stressor

Exposure or threat of death, serious injury, or sexual violence in one or more of the following ways:

- You directly experienced the event.
- You witnessed the event happen to someone else, in person.
- You learned of a close relative or close friend who experienced an actual or threatened accidental or violent death.
- You had repeated indirect exposure to distressing details of the event(s). This could occur in the course of professional duties (first responders, collecting body parts, or professionals repeatedly exposed to details of child abuse). This does not include non-work related exposure through electronic media, television, movies, or pictures.

Criterion B: Intrusion symptoms

The traumatic event is persistently re-experienced in one or more of the following ways:

- Recurrent, involuntary, and intrusive memories. Children older than six may express this symptom through repetitive play in which aspects of the trauma are expressed.
- Traumatic nightmares or upsetting dreams with content related to the event. Children may have frightening dreams without content related to the trauma.
- Dissociative reactions, such as flashbacks, in which it feels like the experience is happening again. These may occur on a continuum ranging from brief episodes to complete loss of awareness. Children may re-enact the events in play.
- Intense or prolonged distress after exposure to traumatic reminders.
- Marked physiological reactivity, such as increased heart rate, after exposure to traumatic reminders.

Criterion C: Avoidance

Persistent effortful avoidance of distressing trauma-related reminders after the event as evidenced by one or both of the following:

- Avoidance of trauma-related thoughts or feelings.
- Avoidance of trauma-related external reminders, such as people, places, conversations, activities, objects, or situations.

Criterion D: Negative alterations in mood

Negative alterations in cognition and mood that began or worsened after the traumatic event as evidenced by two or more of the following:

- Inability to recall key features of the traumatic event. This is usually dissociative amnesia, not due to head injury, alcohol, or drugs.
- Persistent, and often distorted negative beliefs and expectations about oneself or the world, such as "I am bad," or "The world is completely dangerous."
- Persistent distorted blame of self or others for causing the traumatic event or for the resulting consequences.
- Persistent negative emotions, including fear, horror, anger, guilt, or shame.
- Markedly diminished interest in activities that used to be enjoyable.
- Feeling alienated, detached or estranged from others.
- Persistent inability to experience positive emotions, such as happiness, love, and joy.

Criterion E: Alterations in arousal and reactivity

Trauma-related alterations in arousal and reactivity that began or worsened after the traumatic event, including two or more of the following:

- Irritable or aggressive behavior
- Self-destructive or reckless behavior
- Feeling constantly "on guard" or like danger is lurking around every corner (hypervigilance)
- Exaggerated startle response
- Problems in concentration
- Sleep disturbance

Criterion F: Duration

Persistence of symptoms in Criteria B, C, D, and E for more than one month.

Criterion G: Functional Significance

Significant symptom-related distress or impairment of different areas of life, such as social or occupational.

Criterion H: Exclusion

The disturbance is not due to medication, substance use, or other illness.

DSM-5 PTSD Diagnosis

In order to be diagnosed with PTSD according to the DSM-5, you need to meet the following:

- Criterion A
- One symptom or more from Criterion B
- One symptom or more from Criterion C
- Two symptoms or more from Criterion D
- Two symptoms or more from Criterion E
- Criterion F
- Criterion G
- Criterion H

Changes in Diagnostic Criteria

There are a few changes in the latest version of the DSM regarding PTSD diagnosis.3

Key changes include:

- More clearly defining what kind of events are considered traumatic in Criterion A
- Adding a fourth type of exposure in Criterion A

- Increasing the number of symptom groups from three to four by separating avoidance symptoms into their own group (Criterion C)
- Increasing the number of symptoms from 17 to 20
- Changing the wording of some of the symptoms from DSM-IV
- Adding a new set of criteria for children aged 6 or younger
- Eliminating the "acute" and "chronic" specifiers
- Introducing a new specifier "with dissociative symptoms"

Other diagnostic tools

In addition to using the DSM-5 manual to assess for PTSD criteria, a medical professional will likely want to complete a physical examination to check for medical problems that could be contributing to or causing symptoms.

A psychological evaluation is likely to be recommended, which allows for you to openly discuss with your provider some of the events that have led to you experiencing these symptoms. During this evaluation, you would share with a provider signs and symptoms you're experiencing, as well as the duration and level of intensity of these.

This collective information can help medical providers and mental health professionals gain an understanding of your treatment needs and provide you with an appropriate level of care.

Related Conditions

Although the hallmark of post-traumatic stress disorder is having experienced or witnessed a traumatic event, there are a variety of distressing symptoms that someone with PTSD will experience after the event.

Understanding that some of these symptoms may overlap with other mental health conditions, it is important that a thorough evaluation is done to make sure the diagnosis of PTSD is accurate.

Other conditions that could be discussed or explored with your health care provider, due to the symptoms you might be experiencing, could include things like: obsessive-compulsive disorder, acute stress disorder, adjustment disorder, panic disorder, generalized anxiety disorder, major depression, substance abuse, eating disorders.

Again, because some of the distressing symptoms can overlap a bit with other mental health conditions, it is important to talk with a qualified professional to make

sure you receive an accurate diagnosis and are provided with appropriate resources for care and treatment.

Being honest and open with your provider about the symptoms you have been experiencing is critical to helping them understand what is happening for you and to establish the right path for your healing.

(Retrieved from https://www.verywellmind.com/requirements-for-ptsd-diagnosis-2797637)

1. The headline of the article. The author. The publication data.

The article I'm going to give a review of is taken from the website for psychology articles (verywellmind.com.).

The headline of the article is "Symptoms and Diagnosis of PTSD".

It is written by PhD Matthew Tull.

The article is written on the 20th of April in 2021.

2. The topic of the article and logical parts.

The topic of the article is post-traumatic stress disorder.

The purpose of the article is to give the reader some information on criteria for PTSD and PTSD diagnosis.

The article under discussion may be divided into several logically connected parts which are the introduction, the main part and the conclusion. The introduction gives general information about PTSD and the difference between this disorder and stress. Criteria for PTSD and diagnosis are described in the main part. In conclusion the author tells us about the related conditions that can be experienced after a traumatic event.

3. The content of the article. Summary.

The author starts by telling the reader that PTSD is caused by traumatic event.

The author draws reader's attention to the fact that if a person experiences symptoms of PTSD he should have professional help. It is also important to draw a distinction between PTSD and stress.

Much attention is given to criteria for PTSD.

According to DSM there are several criteria for PTSD which are called criterion A – stressor, criterion B – intrusion symptoms, criterion C – avoidance, criterion D – negative alterations in mood, criterion E – alterations in arousal and reactivity, criterion F – duration, criterion G – functional significance, criterion H – exclusion.

The article goes on to say that stressor includes exposure or threat of death, serious injury, or sexual violence which can be experienced by a person, or a person witnesses the event happen to someone else, learns of a close relative or friend who experienced such event(s), faces to the event(s) in the course of professional duties. It is reported that there are intrusion symptoms of PTSD such as recurrent, involuntary, and intrusive memories; traumatic nightmares or upsetting dreams with content related to the event; dissociative reactions, such as flashbacks, in which it feels like the experience is happening again; intense or prolonged distress after exposure to traumatic reminders; marked physiological reactivity, such as increased heart rate, after exposure to traumatic reminders. Criterion C, which is avoidance, includes persistent effortful avoidance of distressing trauma-related reminders (related thoughts or feelings, or

related external reminders, such as people, places, conversations, activities, objects, or situations) after the event. Negative alterations in cognition and mood that began or worsened after the traumatic event, e.g., ersistent, and often distorted negative beliefs and expectations, persistent negative emotions, including fear, horror, anger, guilt, or shame etc., belong to criterion D. Trauma-related alterations in arousal and reactivity that began or worsened after the traumatic event, e.g., irritable or aggressive behavior, self-destructive or reckless behavior, present criterion E. Criterion F is persistence of symptoms in Criteria B, C, D, and E for more than one month. Criterion G includes significant symptom-related distress or impairment of different areas of life, such as social or occupational. Criterion H represents the disturbance is not due to medication, substance use, or other illness.

It is spoken in detail about PTSD diagnosis. In order to be diagnosed with PTSD according to the DSM-5, a person needs to meet the following: criterion A, one symptom or more from criterion B, one symptom or more from criterion C, two symptoms or more from criterion D, two symptoms or more from criterion E, criterion F, criterion G, criterion H. The fact that there are a few changes in the latest version of the DSM regarding PTSD diagnosis is stressed. Key changes are more clearly defining what kind of events are considered traumatic in criterion A; adding a fourth type of exposure in criterion A; increasing the number of symptom groups from three to four by separating avoidance symptoms into their own group (criterion C); increasing the number of symptoms from 17 to 20; changing the wording of some of the symptoms from DSM-IV; adding a new set of criteria for children aged 6 or younger; eliminating the "acute" and "chronic" specifiers; introducing a new specifier "with dissociative symptoms".

Further the author draws the reader's attention to that there are other diagnostic tools such as physical examination and psychological evaluation.

The author comes to the conclusion that there are a variety of distressing symptoms that someone with PTSD will experience after the event such as obsessive-compulsive disorder, acute stress disorder, panic disorder etc. It is important to talk with a qualified professional to make sure you receive an accurate diagnosis because some of the distressing symptoms can overlap a bit with other mental health conditions.

4. Your opinion (conclusion)

Taking into consideration the fact that PTSD is a common disorder and must be corrected this article is currently important.

The main idea of the article is describing criteria for PTSD which is crucial in diagnosing of PTSD.

In addition, some facts about PTSD diagnosis are given.

Back to our main topic it is also important to draw a distinction between PTSD and other mental disorders.

From my point of view the article is easy to understand. The information is well-structured and well-ordered. The vocabulary is vivid and has a lot of terms.

I fully agree with the fact that a person should have professional help if he is experiencing the symptoms of PTSD, and the diagnosis must be accurate in order to be properly treated.

I have found the article of great value because PTSD is a serious mental illness. It is not acceptable to self-medicate because it can lead to after-effects.